

SOFTWARE REQUIREMENTSSPECIFICATION (SRS)

Project Title: MindEase

Empowering Lives Through Accessible, Confidential, and Timely Crisis Response

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Document History

Version	Date	Authors	Change Log	Approved By
V1.0	02.06.2026	Team-	Initial Version	

1. Introduction

1.1 Purpose

This Software Requirements Specification (SRS) document defines the functional and non-functional requirements of **MindEase**, a centralized web-based mental health crisis support platform. It serves as a reference for developers, testers, project managers, administrators, mental health professionals, volunteers, and other relevant stakeholders involved in the development and operation of the system.

1.2 Scope

MindEase is a responsive web-based platform designed to connect individuals experiencing mental health difficulties or crisis situations with trained volunteers, professional psychologists, and relevant support organizations.

The system will allow users to submit crisis support requests, communicate with trained volunteers through secure real-time chat, and receive professional or NGO referrals when required. It will also provide volunteer training and management, volunteer availability and scheduling, a verified mental health resource hub, mental health workshops, and administrative and moderation facilities.

The system will focus on improving accessibility to timely and appropriate mental health support while maintaining user privacy, confidentiality, and responsible service boundaries. MindEase will not provide AI-based diagnosis or counselling, automated medical diagnosis or treatment, native mobile applications, or direct integration with government or emergency-service systems.

1.3 Definitions, Acronyms, and Abbreviations

- **MindEase** – The proposed web-based mental health crisis support platform.
- **SRS** – Software Requirements Specification.
- **UI** – User Interface.
- **Admin** – System administrator responsible for managing and monitoring the platform.
- **NGO** – Non-Governmental Organization.
- **Crisis Request** – A request submitted by an individual seeking mental health or emotional support.
- **Volunteer** – A trained and verified individual who provides peer support to users.
- **Professional** – A verified psychologist who provides professional support and supervises volunteers.
- **RBAC** – Role-Based Access Control.
- **NFR** – Non-Functional Requirement.
- **FR** – Functional Requirement.

1.4 References

- IEEE Std 830-1998 – Software Requirements Specification.
- MindEase Project Proposal, University of Asia Pacific, Version 1.0.
- National Mental Health Survey 2018–19.
- Relevant information regarding existing mental health support services in Bangladesh as identified during project research.

1.5 Overview

This Software Requirements Specification (SRS) document presents the requirements and overall description of the MindEase platform. It provides a structured description of the system to establish a common understanding among the development team, project supervisors, testers, administrators, mental health professionals, volunteers, and other relevant stakeholders.

The document begins with an introduction describing the purpose, scope, terminology, references, and organization of the SRS. The Overall Description section presents the product perspective, major system functions, user characteristics, system constraints, assumptions, and dependencies.

The Specific Requirements section defines the functional and non-functional requirements of MindEase. The functional requirements describe the services and operations that the system shall provide, including user authentication, crisis support requests, volunteer-based peer support, secure communication, professional and NGO referrals, volunteer management and scheduling, mental health resources, workshops, and administration and moderation. The non-functional requirements describe the expected qualities of the system, including performance, security, privacy, usability, reliability, availability, maintainability, and data integrity.

The External Interface Requirements section describes how users and external components will interact with the system through user interfaces, hardware, software, and communication interfaces. The SRS also includes system models and supporting documentation to provide a clearer representation of the system's processes, interactions, and data requirements.

Finally, the document includes stakeholder survey and interview questionnaires, demographic information, collected responses, analysis of the findings, and the resulting requirements identified through the requirement elicitation process. These findings are used to validate and refine the requirements of MindEase and ensure that the proposed system addresses the needs of its intended users and stakeholders.

2. Overall Description

2.1 Product Perspective

MindEase is a centralized web-based platform intended to bring several mental health support functions together within a single system. Instead of relying on fragmented services, the platform will connect support seekers with trained volunteers and, when necessary, facilitate escalation to professional psychologists or referral to relevant NGOs.

The system will also provide supporting functions such as volunteer management, scheduling, training, verified mental health resources, workshops, and administrative moderation.

The platform will use a **multi-role architecture**, where different users receive access to functions appropriate to their responsibilities. The main user groups are support seekers, trained volunteers, professional psychologists, partner NGOs, workshop doctors/facilitators, and administrators.

2.2 Product Functions

The major functions of MindEase include:

1. **User Authentication and Access Control** – Provides secure authentication and role-based access.
2. **Crisis Request Management** – Allows users to submit requests for support.
3. **Volunteer Support** – Connects users with available trained volunteers.
4. **Professional Referral** – Allows volunteers to escalate appropriate cases to psychologists.
5. **NGO Referral** – Allows relevant cases, such as domestic violence or abuse, to be referred to partner NGOs.
6. **Secure Real-Time Communication** – Enables encrypted communication between users and support providers.
7. **Volunteer Training and Management** – Supports volunteer verification, training status, availability, scheduling, and management.
8. **24/7 Volunteer Coverage Management** – Supports scheduling and availability management to maintain continuous coverage.
9. **Verified Mental Health Resource Hub** – Provides verified information, helplines, emergency resources, and relevant organizations.
10. **Mental Health Workshop Management** – Allows workshops to be created, managed, browsed, registered for, and paid for.
11. **Professional Video Consultation** – Allows psychologists to conduct video consultations when appropriate.
12. **Administration and Moderation** – Allows administrators to manage users, professionals, volunteers, NGOs, workshops, reports, and platform activities.

2.3 User Classes and Characteristics

User	Description
Individuals Seeking Support	These are the primary end-users of MindEase. They may have emotional difficulties, mental health concerns, or crisis situations. They should be able to use basic web and communication functions to submit support requests, communicate with volunteers, access resources, and receive referrals
Trained Volunteers	Volunteers provide peer support to individuals experiencing distress. They are expected to have completed the required training and verification process. Volunteers should be able to manage their availability, accept support sessions, communicate with users, follow professional guidance, and escalate cases when necessary.
Professional Psychologists	Psychologists provide professional support to referred users and supervise or guide trained volunteers. They should be able to review appropriate referrals, communicate with users, provide guidance, and conduct video consultations where applicable
Partner NGOs	Partner NGOs provide specialized social support for cases such as domestic violence, abuse, or other social issues. They should be able to receive relevant referrals and manage the cases assigned to them.
Workshop Doctors and Facilitators	These users conduct mental health awareness and educational workshops. They should be able to manage workshop-related information and provide expert guidance for workshop content and delivery.
Moderator / Administrator	Administrators are responsible for overall platform management and moderation. They manage accounts, verify users and support providers, manage workshops and resources, monitor reported content, and oversee platform activities.

2.4 Operating Environment

MindEase will operate as a web-based application accessible through modern web browsers on desktop, laptop, tablet, and smartphone devices. The system will require an active internet connection for user authentication, crisis support requests, real-time communication, referrals, workshops, and access to mental health resources.

The application will operate on a web server connected to a database server for storing and managing user, support, referral, volunteer, resource, and workshop information. The system will be designed to support commonly used operating systems and modern browsers without requiring users to install a separate application.

2.5 Design and Implementation Constraints

The following constraints shall be considered during the design and implementation of MindEase:

- The system shall be implemented as a **responsive web-based platform**.
- Native Android and iOS applications are outside the current project scope.
- The system shall **not use Artificial Intelligence (AI) or Machine Learning (ML)** for diagnosis, counselling, or crisis assessment.
- The system shall not provide automated medical diagnosis, treatment, or medication recommendations.
- Sensitive communication and user information shall be protected using appropriate security and encryption mechanisms.
- Role-based access control shall be used to restrict access to information according to user responsibilities.
- Volunteers shall provide peer support and shall not perform professional diagnosis or treatment.
- Professional psychological support shall be provided only by verified psychologists.
- Direct integration with government or emergency-service systems is outside the current scope.
- The implementation must remain feasible within the project's available time, resources, and technical capabilities.

These constraints are consistent with the defined scope and exclusions of the MindEase proposal.

2.6 Assumptions and Dependencies

The following assumptions and dependencies apply to the MindEase system:

- Users will have access to an internet-connected device and a modern web browser.
- Users will provide accurate information when registering or submitting support requests.
- Volunteers will complete the required training and verification process before providing peer support.
- Psychologists participating in the platform will be appropriately verified.
- Partner NGOs will provide appropriate support for cases referred to them.
- Administrators will regularly review and update verified mental health resources.
- Volunteers will update their availability so that the scheduling system can assign support sessions appropriately.
- Users requiring professional intervention will be referred to appropriate psychologists or organizations.
- The availability of volunteers and partner organizations may affect the level and timing of support provided through the platform.

The continued operation of the platform may depend on partnerships, sponsorships, grants, and revenue from paid workshops.

3. Specific Requirements

3.1 Functional Requirements

3.1.1 User Authentication & Access Control

- **FR 1.1 –** The system shall allow users to create an account using the required registration information. (High)
- **FR 1.2 –** The system shall allow registered users to securely log in and log out. (High)
- **FR 1.3 –** The system shall provide role-based access according to the user's assigned role. (High)
- **FR 1.4 –** The system shall restrict users from accessing functions that are not permitted for their assigned role. (High)

3.1.2 Crisis Request & Peer Support

- **FR 2.1 –** The system shall allow individuals seeking support to submit a crisis support request. (High)
- **FR 2.2 –** The system shall record and manage submitted crisis requests. (High)
- **FR 2.3 –** The system shall allow available trained volunteers to accept assigned crisis requests and communicate with users. (High)

3.1.4 Professional Referral & Consultation

- **FR 4.1 –** The system shall allow volunteers to escalate cases requiring professional intervention to verified psychologists. (High)
- **FR 4.2 –** The system shall allow verified psychologists to view cases referred to them. (High)
- **FR 4.3 –** The system shall allow psychologists to communicate with referred users through the platform. (High)
- **FR 4.4 –** The system shall allow psychologists to conduct video consultations with users when required. (Medium)

3.1.5 NGO Referral

- **FR 5.1 –** The system shall allow volunteers to refer appropriate cases, including domestic violence or abuse cases, to verified partner NGOs. (High)
- **FR 5.2 –** The system shall allow authorized NGO representatives to view cases referred to their organization. (High)
- **FR 5.3 –** The system shall allow authorized NGO representatives to update the status of referred cases. (Medium)

3.1.6 Volunteer Management & 24/7 Coverage

- **FR 6.1 –** The system shall allow administrators to verify volunteer accounts before they provide peer support. (High)
- **FR 6.2 –** The system shall allow administrators to manage volunteer training and verification status. (High)
- **FR 6.3 –** The system shall allow volunteers to update their availability status. (High)
- **FR 6.4 –** The system shall allow administrators to create and manage volunteer schedules to support continuous coverage. (High)

3.1.7 Mental Health Resource Hub

- **FR 7.1 –** The system shall provide users with a centralized collection of verified mental health resources. (High)
- **FR 7.2 –** The system shall provide relevant helpline, emergency-resource, and support-organization information. (High)
- **FR 7.3 –** The system shall allow administrators to add, update, and remove resources. (Medium)

3.1.8 Workshop Management

- **FR 8.1 –** The system shall allow authorized facilitators or administrators to create and manage mental health workshops. (Medium)
- **FR 8.2 –** The system shall allow users to view and register for available workshops. (Medium)

- **FR 8.3 –** The system shall support payment processing for paid mental health workshops. (Medium)

3.1.9 Administration & Moderation

- **FR 9.1 –** The system shall allow administrators to manage user and stakeholder accounts. (High)
- **FR 9.2 –** The system shall allow administrators to verify psychologists, NGOs, volunteers, and other relevant stakeholders. (High)
- **FR 9.3 –** The system shall allow administrators to monitor reported content and overall platform activities. (Medium)

3.2 Non-Functional Requirements

3.2.1 Performance Requirements

- **NFR 1.1 –** The system shall load standard web pages within 3 seconds under normal network conditions. (High)
- **NFR 1.2 –** The system shall support at least 200 concurrent users without significant degradation in performance. (Medium)

3.2.2 Security & Privacy Requirements

- **NFR 2.1 –** The system shall encrypt sensitive communication between users, volunteers, psychologists, and other authorized stakeholders. (High)
- **NFR 2.2 –** The system shall protect stored sensitive user information using appropriate security mechanisms. (High)
- **NFR 2.3 –** The system shall implement role-based access control to restrict access to sensitive information according to user responsibilities. (High)
- **NFR 2.4 –** The system shall securely authenticate users before granting access to protected system functions. (High)

3.2.3 Usability Requirements

- **NFR 3.1 –** The system shall provide a simple and intuitive user interface that can be used by users with basic web browsing knowledge. (High)
- **NFR 3.2 –** The system shall provide clear and understandable error messages when invalid information is entered. (Medium)
- **NFR 3.3 –** The system shall provide a responsive interface that can be accessed through commonly available desktop and mobile web browsers. (High)

3.2.4 Reliability & Availability Requirements

- **NFR 4.1 –** The system shall maintain an availability of at least 99% during its operational period. (High)
- **NFR 4.2 –** The system shall perform regular backups of critical system and user data to minimize data loss. (High)
- **NFR 4.3 –** The system shall recover from a system failure without corrupting previously stored user or case information. (High)

3.2.5 Maintainability Requirements

- **NFR 5.1 –** The system shall use a modular architecture so that individual components can be maintained or updated without significantly affecting other components. (Medium)
- **NFR 5.2 –** The system shall follow standard software development and documentation practices to facilitate future maintenance. (Medium)

3.2.6 Data Integrity

- **NFR 6.1 –** The system shall maintain the accuracy and consistency of user, support-request, referral, volunteer, and workshop data during storage and processing. (High)

4. External Interface Requirements

4.1 User Interfaces

- The system shall provide a responsive web-based graphical user interface accessible through desktop, laptop, tablet, and smartphone browsers.
- The system shall provide separate dashboards and interfaces according to the user's assigned role.
- The system shall provide simple forms for user registration, crisis requests, referrals, workshop registration, and other required data entry.
- The system shall provide an interface for real-time communication between support seekers, volunteers, and authorized professionals.
- The system shall provide dashboards for volunteers to view assigned cases, manage availability, and access relevant support information.
- The system shall provide dashboards for psychologists and NGOs to manage referred cases and related activities.

- The system shall provide an administrative dashboard for user management, verification, moderation, resources, workshops, and system activities.
- The system shall provide clear navigation and understandable messages to support users with different levels of digital literacy.

4.2 Hardware Interfaces

- The system shall operate on standard computers, laptops, tablets, and smartphones capable of running a modern web browser.
- The system shall require an internet-connected client device for accessing the web application.
- The system shall support microphone and camera access on compatible devices when a psychologist conducts a video consultation.
- No specialized hardware shall be required for normal operation of the system.

4.3 Software Interfaces

- The system shall communicate with a database system for storing and retrieving user, support, volunteer, referral, resource, and workshop information.
- The system shall use secure web communication protocols for communication between the client application and server.
- The system shall support integration with a video communication service or SDK for psychologist video consultations.
- The system shall support integration with a payment service for paid mental health workshops.
- The system shall support modern web browsers including commonly used versions of Chrome, Firefox, Edge, and Safari.
- The system shall support backend APIs required for communication between the web interface, application logic, and database.

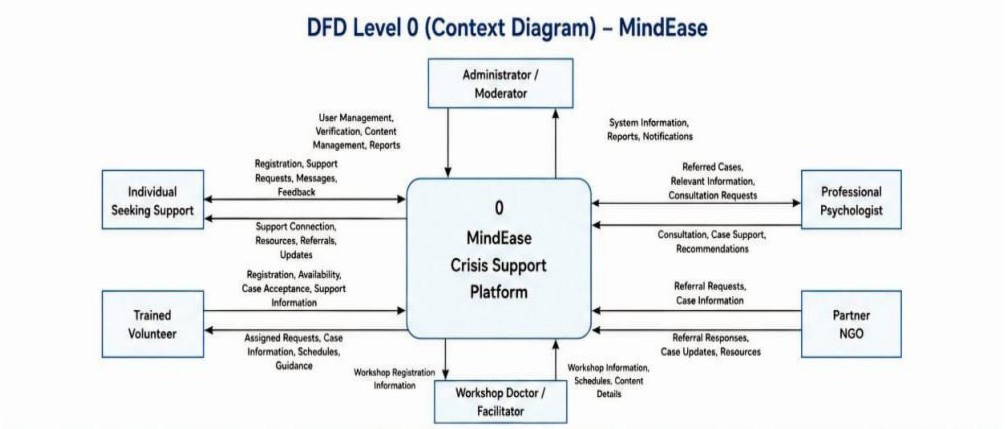
4.4 Communication Interfaces

- The system shall use HTTPS for secure communication between users' browsers and the server.
- The system shall provide encrypted real-time communication for sensitive conversations between authorized users.
- The system shall support real-time data exchange required for chat and video consultation features.
- The system shall provide appropriate session management for authenticated users.
- The system shall restrict communication and exchanged information according to user roles and case assignments.

5. System Models

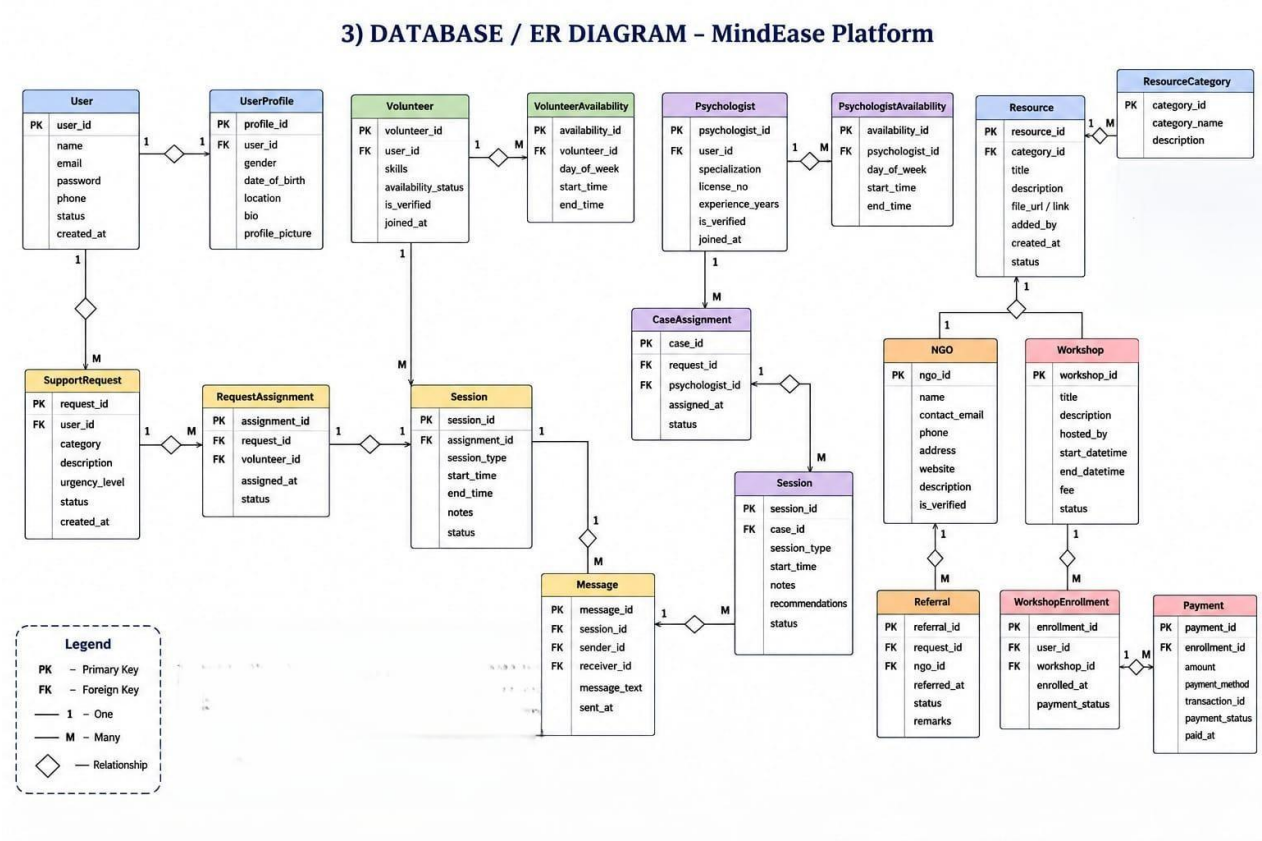
The following models will be produced during the design phase and maintained as living artifacts:

- **Use Case Diagram** – capturing interactions among Support Seekers (Primary End-Users), Trained Volunteers, Professional Psychologists, Partner NGOs, and System Administrators.
- **Data Flow Diagram (DFD)** – Level 0 (Context Diagram) and Level 1 for crisis request handling, risk escalation, peer chat routing, and workshop payment processing flows.
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6. Other Requirements

6.1 Data and Privacy Requirements

- The system shall collect only the personal and case information necessary to provide support and manage referrals.
- The system shall obtain appropriate user consent before collecting or sharing sensitive information.
- The system shall protect confidential user information from unauthorized access.
- The system shall restrict access to sensitive information according to the responsibilities of each stakeholder role.
- Information shared during referrals to psychologists or NGOs shall be limited to information necessary for providing the required support.

6.2 Ethical and Service Boundary Requirements

- The system shall clearly communicate that volunteer support is peer support and does not replace professional mental health treatment.
- Volunteers shall not diagnose mental health conditions or prescribe treatment.
- Only verified psychologists shall provide professional mental health support through the platform.
- Cases requiring professional intervention shall be appropriately escalated to qualified professionals.
- The system shall not provide AI-based diagnosis, automated crisis assessment, AI counselling, or AI chatbot services.
- The system shall not provide automated medical diagnosis, medication prescriptions, or automated medical treatment.

6.3 Resource and Content Requirements

- Mental health resources provided through the platform shall be reviewed and verified before being made available to users.
- Administrators shall be able to update outdated or incorrect resource information.
- Workshop and support information shall be maintained so that users receive current information.
- The system shall provide relevant helplines, mental health information, and support-organization information through the centralized resource hub.

6.4 Accessibility and Sustainability Requirements

- The system shall use a simple and responsive design to support access through commonly available devices.
- The system shall consider users with limited digital literacy when designing forms, navigation, and system messages.
- Volunteer availability and workload shall be managed to reduce excessive workload and support continuous coverage.
- Paid workshops, NGO partnerships, sponsorships, and grants may be used as potential mechanisms for supporting the long-term sustainability of the platform.

7.Requirements Traceability

Req ID	Requirement (Summary)	Source	Priority	Acceptance Criteria (High-level)
FR 1.1	Account Registration	User Survey + Data Privacy	High	Registration completes using required info and role-based access is set.
FR 2.1	Submit Crisis Request	Problem Validation + Survey	High	System allows support seekers to submit crisis requests to the active queue.
FR 2.3	Peer Support Chat	Problem Validation + Field Research	High	Assigned trained volunteer accepts request and communicates with user via platform.
FR 4.1	Escalation to Psychologists	Clinical Input + Stakeholder Interviews	High	Volunteers can successfully escalate high-risk cases to verified psychologists.
FR 5.1	Partner NGO Referral	Institutional Input + Survey	High	System routes appropriate cases (e.g., domestic violence) to partner NGOs.
FR 8.3	Workshop Payment Processing	Student Survey (Pricing & Workshops)	Medium	System processes payment successfully for paid mental health workshops.
NFR 1.1	Web Loading Performance	System Performance Benchmark	High	Standard web pages load within 3 seconds under normal network conditions.

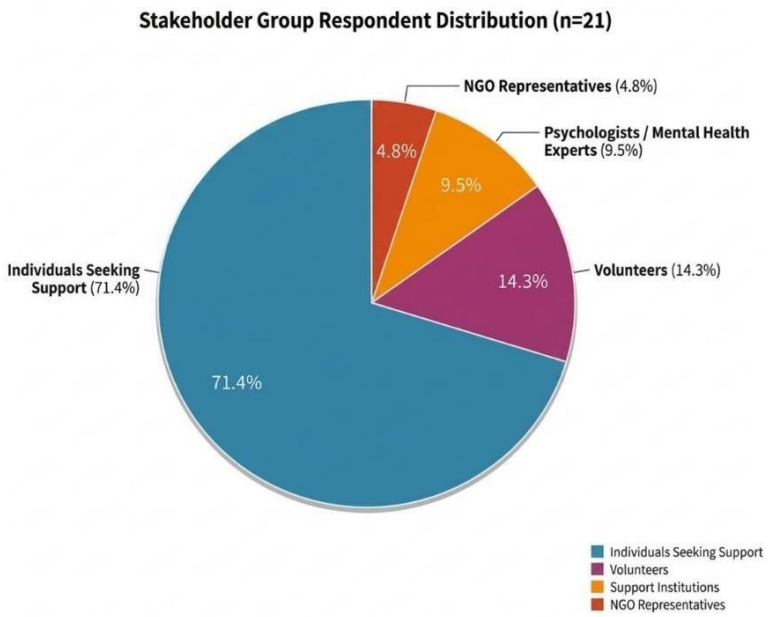
NFR 2.1	Sensitive Data Encryption	Security & Ethical Analysis	High	System encrypts sensitive communication between users, volunteers, and psychologists.

Appendix A: Requirement Elicitation Process

Requirements were obtained through a structured questionnaire survey involving four stakeholder groups that interact with mental health crisis support from different perspectives. A total of 15 completed responses were received and analyzed.

MindEase – Software Requirements Specification

Stakeholder Group	Responses	% of Total
Individuals Seeking Support	15	71.4%
Volunteers	3	14.3%
Psychologists / Mental Health Experts	2	9.5%
NGO Representatives	1	4.8%
Total	21	**100%**



Appendix B: Survey Responses and Statistics

B.1 Overall Picture of Responses

Individuals seeking support formed the largest share of respondents (71.4%). Peer volunteers accounted for 14.3%, mental health experts / psychologists contributed 9.5%, and NGO representatives represented 4.8%. The distribution reflects the primary focus on end-user crisis support needs while still incorporating initial responder, clinical, and institutional viewpoints.

Steps Followed:

1. **Stakeholder identification** – Four groups were selected: primary users (individuals seeking support), people who provide initial assistance (volunteers), mental health professionals (psychologists), and partner organizations (NGOs).
2. **Questionnaire design** – Separate question sets were prepared for each group, covering mental health concerns, anonymous communication preferences, crisis request handling, case escalation, resource availability, offline/low-bandwidth constraints, and privacy expectations.
3. **Data collection** – Responses were obtained directly from members of the four groups.

4. **Analysis** – Completed forms were examined for repeated concerns, preferred actions in mental health crises, and stated expectations of a digital crisis support platform.
5. **Requirement formulation** – Similar answers were clustered. Themes that appeared across more than one group were retained as candidate requirements.
6. **Prioritization** – Needs voiced by the primary user group (support seekers) and reinforced by clinical or institutional stakeholders were ranked higher.
7. **Cross-check** – Each candidate requirement was compared against responses from multiple groups before being accepted into the SRS.
8.

B.1 Overall Picture of Responses

Individuals seeking support formed the largest share of respondents (46.7%). Volunteers and psychologists each contributed 20%, and partner NGO representatives 13.3%. The distribution reflects the primary focus on end-user needs while still incorporating professional and institutional viewpoints.

Most requirements (8 out of 10) were independently mentioned by two different stakeholder groups. One requirement appeared in three groups, and only one was raised by a single group. This pattern was used as an indicator of broader relevance.

B.2 Observations from Individuals Seeking Support

Support seekers frequently referred to psychological and situational cues—such as immediate distress levels, fear of judgment, and past negative experiences—when deciding whether to seek mental health assistance. Many also indicated they would prefer to request support anonymously without revealing their personal identity or contact details. Several described situations in which stable internet connectivity might be limited or interrupted and still expected to access basic offline resources or emergency crisis numbers. Interest was expressed in a quick way to initiate continuous chat support and in knowing the locations and contact details of verified psychological services, emergency helplines, and partner organizations.

1. **What is your age?**
    - 16-20
    - 21-25
    - 26-30
    - 30+
2. **What is your current status?**
    - University student
    - Working professional
    - Both studying and working

3. **Do you struggle with any mental health challenges?**
    - Anxiety
    - Depression
    - Stress
    - Sleep issues
    - Exam pressure
    - Relationship issues
    - None / Don't know
    - Prefer not to say
4. **Have you ever sought help for mental health?**
    - Yes, from a professional
    - Yes, from friends/family
    - No, but I wanted to
    - No, didn't feel the need
    - Don't know where to get help
5. **What do you think is the biggest barrier to getting mental health help in Bangladesh?**
    - Cost
    - Stigma/shame
    - Don't know where to go
    - Fear of being judged
    - Long waiting times

6. **How would you prefer to access the platform when seeking support?**
    - Completely anonymously
    - Using a display name / pseudonym
    - By creating a registered account with my real details
7. **Which data privacy feature is most important to you when sharing personal mental health concerns?**
    - End-to-end encrypted chat messages
    - Assurance that my data is never shared with third parties
    - Option to delete my chat history and account at any time

8. **During emotional distress, how would you prefer to initiate contact with a volunteer?**
    - Instant one-click chat request
    - Filling out a short form (selecting mood/topic) before connecting
    - Direct hotline / emergency call option
9. **What communication channels would you feel most comfortable using with a trained volunteer?**
    - Real-time Text Chat
    - Voice Call
    - Video Call

10. **If a volunteer suggests escalating your case to a professional psychologist, how should the system handle it?**
    - Automatically refer me and provide an appointment booking link
    - Ask for my explicit consent before displaying available psychologists

- Provide suggestions only; let me search and book independently

11. **In situations involving domestic violence or abuse, what assurance or process do you require before being referred to a partner NGO?**
    - Explicit consent required before sharing any personal data
    - Full transparency on what specific data will be shared with the NGO
    - Option to remain anonymous to the NGO until I choose to share identity
    - Clear details on what safety, shelter, or legal support the NGO provides

12. **What resources would you like to access in the platform's Resource Hub?**
    - 24/7 Emergency Helplines
    - Articles/Guides on managing stress, anxiety, and depression
    - Directory of verified mental health centers and hospitals
    - Self-help audio or video guides
13. **How would you prefer to track your ongoing support or consultation history?**
    - A private dashboard saved to my profile
    - Temporary session links sent via email/SMS (no history stored)
    - Option to download a password-protected summary after each session
14. **What is your primary expectation or requirement from this crisis support platform?**

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B.3 Observations from Volunteers

Trained volunteers who are called upon to provide peer support and crisis assistance emphasised the importance of receiving prompt notification when a crisis support request is submitted, accessing real-time case updates and risk indicators, being able to communicate with the support seeker in a secure and confidential environment, and having a clear mechanism to escalate high-risk cases to psychologists or partner NGOs when needed.

1. **What is your age?**
    - 18-25
    - 25-30
    - 30-40
    - 40+
2. **Do you have any background in mental health or counseling?**
    - Yes, professional training (degree/certification)
    - Yes, some training (workshops, courses)
    - Yes, personal experience helping others
    - No, but interested to learn
    - No
3. **Have you volunteered before?**

- Yes, in mental health
- Yes, in other fields
- No, but interested

4. **Which scheduling tools would best help you manage your availability and prevent volunteer burnout?**

- Flexible shift booking system (e.g., selecting specific hours/slots)
- Automated notifications for upcoming active shifts
- Ability to set maximum continuous hours per day/week
- "Status toggle" (Available, Busy, Offline) for instant availability management

5. **What essential features do you need during an active real-time support session with a user?**

- Pre-approved crisis-response templates / quick-reply guides
- Real-time mood / crisis severity indicator (e.g., Low, Moderate, High risk)
- Direct access to a supervisor / psychologist via a private internal chat
- Easy access to verified helpline lists and self-help resources within the chat interface

6. If you were to volunteer, what do you primarily expect to gain or learn from the mental health training workshops hosted on the platform?

- Practical crisis-intervention protocols and active listening techniques
- Guidance from professional psychologists on managing severe or high-risk cases
- Knowledge on identifying when and how to escalate cases to doctors or NGOs
- Strategies for managing volunteer stress, burnout, and maintaining personal well-being
- Official training certificates and skill badges upon completion

7. When you identify a case requiring professional psychological intervention, how should the platform facilitate escalation?

- One-click escalation button that flags the session and notifies an available psychologist
- Filling out a quick escalation summary form before routing the user
- Transferring the user to a dedicated professional queue with explicit user consent

8. In cases involving domestic violence, abuse, or social issues, what process do you need for NGO referrals?

- Pre-categorized directory of verified partner NGOs based on specific crisis types
- System-generated referral ticket that tracks the status of the NGO response
- Option to attach session summaries securely while maintaining user privacy preferences

9. How should guidance and supervisor training from professional psychologists be integrated into your workflow?

- Mandatory periodic refresher training modules hosted on the platform
- Direct feedback/review from psychologists on selected escalated cases
- Centralized repository of psychologist-approved crisis guidelines and protocols

10. What tools would best support your personal mental well-being and stress management as a volunteer?

- Debriefing sessions / support groups with psychologists
- Automatic cool-down periods required between high-intensity crisis sessions
- Easy option to temporarily freeze profile / take leave for mental health rest

11. How would you prefer to receive shift updates, platform announcements, or supervisor feedback?

- In-app notification center / dashboard alerts
- Direct email notifications
- Instant SMS or mobile alerts for urgent shift updates

12. Which analytical or reporting tools would help you monitor your peer-support activities?

- A personal dashboard showing completed support sessions and total hours logged
- Option to review past case summaries and supervisor ratings/feedback
- Performance statistics (e.g., average response time, user satisfaction feedback)

13. Which primary communication channels should be available to you to effectively assist users?

- Real-time Text Chat only
- Real-time Text Chat and Voice Call options
- Real-time Text Chat, Voice Call, and Video Call options

14. What is your primary requirement or expectation from the platform to carry out your role effectively and safely?

_____

_____

_____

B.4 Observations from Psychologists / Mental Health Experts

Psychologists and mental health experts focused on the clinical accuracy and risk assessment of information shared within the platform. They indicated that user-reported distress levels and escalation flags should be corroborated through structured risk screening rather than accepted

without checks. This theme contributed to the inclusion of verification mechanisms and standardized escalation protocols.

1. How would you prefer your credentials and professional license to be verified before taking client referrals?

- Document upload (BMDC license, degrees) verified by platform admins
- Direct verification interview with lead psychologists
- Institutional verification through partner organizations

2. Which scheduling tools do you require to manage your availability effectively?

- Customizable calendar for setting available slots
- Option to limit daily crisis escalations/appointments
- Instant toggle switch to mark status (Available/Busy)
- Automated reminders before scheduled consultations

3. When a volunteer escalates a high-risk case, what information do you require in the initial handover?

- Brief crisis summary written by the volunteer
- Anonymized chat log between user and volunteer
- Risk assessment indicator (Low/Moderate/High)
- User's basic demographic background

4. What communication channels do you require for conducting professional assessment sessions?

- Secure, encrypted video consultation
- Encrypted voice call
- High-security text-based consultation

5. How would you prefer to conduct or oversee volunteer training and crisis response guidelines?

- Live interactive workshops and Q&A webinars
- Uploading recorded lectures and manuals to a resource hub
- Reviewing and approving crisis response templates
- Assigning periodic assessment quizzes

6. What mechanisms do you prefer for supervising volunteers during active crisis situations?

- Private supervisor chat for real-time guidance
- Automated flagging system for high-risk keywords
- Post-session review system for flagged logs

7. In complex cases involving social issues or abuse, how should referrals to partner NGOs be coordinated?

- Direct transfer to verified NGOs with attached clinical notes
- Joint referral requiring consent from both user and psychologist

- Formal referral summary given to the user to present independently

8. What tools do you need for maintaining confidential client notes and ongoing treatment history?

- Encrypted private dashboard for saving progress notes
- Option to generate password-protected summaries for users
- Automated data-purge feature after a retention period

9. What resources would you like to contribute to or manage within the platform's Resource Hub?

- Publishing verified psychoeducational articles and guides
- Reviewing and approving emergency helpline directories
- Creating stress management audio/video content

10. What metrics would be most useful on your professional dashboard?

- Total number of handled escalations and consultations
- Overview of volunteer supervision metrics
- User feedback and session outcome ratings

11. Which data security and privacy measures are essential for you on this platform?

- End-to-end encryption for consultations and chat logs
- Strict role-based access control for medical notes
- Full compliance with medical data confidentiality standards

12. What is your primary requirement from this platform to provide effective psychological support?

B.5 Consolidated Requirements

After clustering and cross-checking the responses, the following set of requirements was retained:

1. Anonymous crisis request reporting
2. Immediate emergency escalation trigger
3. Managed queue of peer volunteers and psychologists
4. Referral system to verified partner NGOs
5. Secure payment processing for mental health workshops
6. End-to-end data encryption and fast platform response times

Taken together, the survey points toward a system whose main value lies in rapid crisis response, privacy and anonymity, seamless escalation to expert psychologists, and secure communication with verified support providers.

Appendix C: Requirement Elicitation

Reference: Stakeholder Groups

Stakeholder Group	Role in Ecosystem
Individuals Seeking Support (Primary End-Users)	Individuals, students, and working professionals relying on the platform for proactive mental health guidance, anonymous crisis support, and emergency escalation.
Volunteers (Initial Responders)	Trained peer support volunteers who monitor active queues, conduct initial risk assessments, and provide confidential chat assistance.
Psychologists / Mental Health Experts	Verified professional psychologists who handle escalated high-risk cases and provide clinical oversight.

Support Institutions / Partner NGOs	Mental health organizations, emergency helplines, and specialized NGOs (e.g., domestic violence support) providing specialized intervention.
Development / Project Team	Project lead, system architect, UI/UX designer, and software developers managing platform implementation and maintenance.

Individuals Seeking Support

1. Overview

1. **Interviewee(s):** Primary user respondents (individuals seeking mental health support)
2. **Interviewer(s):** Nusrat Jahan Tripty, Anisur Rahman Sehzan, Mehrima Mim, Sawda Rahman, Siam , Khadiza Manha, Sadia Islam Usha
3. **Purpose of Interview:** Understand individuals' experiences, concerns, and needs when seeking mental health support in Bangladesh—especially in situations involving emotional distress or crisis—to inform the design of MindEase, a privacy-centric crisis support platform. The survey focused on support-seeking factors, desired mental health resources, barriers to seeking help, emergency escalation preferences, privacy requirements, offline capabilities, and features that would make crisis intervention accessible and stigma-free.

2. Key Objectives Discussed

- Factors that influence the decision to seek peer support or professional therapy.
- Information, self-help tools, or warnings that could help de-escalate emotional distress.
- Conditions under which professional therapy would be preferred over peer volunteer chat.
- Barriers to reporting mental health crises and the role of absolute anonymity.
- Preferred ways to reach emergency crisis lines under poor signal, no data, or offline conditions.
- Who should be notified in critical risk scenarios and what information they should receive.
- Preferences for crisis follow-ups and knowledge of nearby verified mental health centers or NGOs.
- Level of trust in community-sourced peer support versus verified professional advice.
- Impact of mental health stigma on seeking timely psychological support.
- Features that would make a crisis support tool genuinely effective in a real crisis.

3. Findings / Key Points

1. Uncertainty and fear of judgment when reaching out for mental health support, particularly during severe crisis episodes.

2. Lack of clear, real-time access to verified resources (peer availability, professional consultation slots, specialized NGO support).
3. Strong reluctance to seek help or disclose distress due to fear of identity exposure, social stigma, and privacy breaches.
4. Inability to effectively access online support during network blackouts, poor signal, or unavailable mobile data.
5. Having previously delayed or avoided seeking professional therapy due to social stigma, lack of anonymous options, or high costs.

Evidence



Trained Volunteers

1. Overview

1. **Interviewee(s):** Peer support volunteers and community mental health responders (3 total responses)
2. **Interviewer(s):** Rupayan Barua, Sunandan Das, Md. Hafiz Tamim, Ameer Khasru
3. **Purpose of Interview:** Understand what makes peer support volunteers more likely to trust incoming crisis requests, what operational barriers they face regarding initial chat handling, risk triage, and escalation decisions, and how to maintain secure, high-quality peer interactions on the MindEase platform.

2. Key Objectives Discussed

The survey focuses on volunteer-driven initial response and crisis triage systems, with these core objectives:

- Assessing trust in user-submitted distress indicators and the impact of inaccurate or exaggerated support requests.
- Identifying factors that make user crisis requests credible and actionable (e.g., self-assessed severity levels, initial distress screening, anonymity maintenance).
- Understanding willingness to accept, handle, or escalate crisis chat sessions, and what motivates continuous volunteer participation.
- Determining how peer volunteer initial triage should be weighed against formal clinical assessments by professional psychologists.
- Identifying barriers to keeping peer responder queues active, confidential, and up to date.
- Exploring mechanisms for handling abusive, fake, or non-genuine support requests (e.g., volunteer reporting flags, automated rate-limiting).
- Balancing user anonymity with internal volunteer accountability and platform safety guidelines.
- Designing low-burden crisis intake processes and seamless handoff protocols for professional escalation.
- Filtering spam or abusive users without discouraging genuine support-seeking individuals.
- Evaluating preference for dynamic, real-time crisis queue management versus static request logs.

Overall aim: Inform a system that supports anonymous, safe, continuous, and verified peer support interaction while remaining practical, clinically safe, and trustworthy for users in Bangladesh.

3. Findings / Key Points

1. **Structured Anonymity & Risk Triage:** Strong preference for anonymous peer chat backed by initial distress screening over unverified open messaging or mandatory identity disclosure.
2. **Dynamic Queue Management:** Real-time crisis request queues driven by active volunteer availability and automated distress prioritization are positioned as far more responsive than static request logs.
3. **Low-Friction Escalation:** Simple, low-friction mechanisms (one-tap escalation to psychologists, standardized risk flags, concise session summaries) are favored to ensure rapid intervention in critical cases.
4. **Volunteer Retention Barriers:** Operational hurdles such as complex session logging, lack of clear clinical boundary guidelines, and unmitigated spam requests are primary obstacles to volunteer retention.
5. **Accountability & Safety:** Volunteer activity can be monitored through internal supervisor ratings and trust badges without compromising the end-user's complete identity privacy.

6. **Abuse Prevention:** Spam and non-genuine requests should be managed via automated rate-limiting, system filters, and peer flagging rather than enforcing real-name identity disclosure on distressed users.
7. **Integrated Handoff:** Peer support acts as an essential initial triage layer that seamlessly connects high-risk users to verified psychologists and partner NGOs rather than operating as a standalone tool.

Evidence



Psychologists & Mental Health Experts

1. Overview

1. **Interviewee(s):** Licensed psychologists and clinical mental health experts (3 total responses)
2. **Interviewer(s):** Shamsunnaher Alo(Restart-Phycological Consultancy
3. **Purpose of Interview:** Understand how professional psychologists evaluate risk

indicators in digital mental health platforms, identify clinical barriers in remote crisis intervention, establish clinical handoff protocols from peer volunteers, and determine requirements for verified appointment scheduling, session confidentiality, and crisis escalation within the **MindEase** platform.

2. Key Objectives Discussed

The survey focuses on expert-level clinical oversight, structured risk assessment, and professional consultation workflows, with these core objectives:

- Evaluating trust in preliminary distress scores provided by users or initial peer volunteer intake notes.
- Establishing standardized clinical criteria for escalating high-risk cases (e.g., self-harm indicators, severe depression, acute anxiety) from peer chat to professional sessions.
- Determining privacy and identity protection protocols that maintain clinical ethics while preserving user anonymity.
- Identifying necessary dashboard features for managing escalated crisis queues, appointment schedules, and client case summaries.
- Evaluating requirements for paid clinical workshop management, consultation verification, and session availability listing.
- Designing low-friction, secure communication channels for conducting digital consultations without compromising patient data confidentiality.
- Defining protocols for transferring severe critical cases to partner mental health NGOs, crisis centers, or emergency hospital units.
- Mitigating clinical liability concerns regarding unverified public health recommendations or non-genuine requests.

Overall aim: Inform a robust clinical workflow within MindEase that ensures high-risk users receive verified professional care through secure, ethically compliant, and seamless escalation protocols.

3. Findings / Key Points

1. **Clinical Risk Validation:** Strong preference for structured clinical screening over user self-diagnosis, requiring standardized distress metrics before accepting escalated consultations.
2. **Seamless Handoff Protocols:** Essential need for automated notification flags and summarized peer-intake history when a case is escalated from a volunteer to a professional psychologist.
3. **Flexible Anonymity with Ethical Safeguards:** End-user identity privacy should be maintained during initial sessions, with clinical mechanisms to request emergency contact info only in life-threatening scenarios.
4. **Structured Availability & Booking:** High preference for dynamic consultation slot scheduling and secure processing for paid mental health workshops to prevent double-booking and off-hour access issues.
5. **Clear Professional Boundaries:** Strong emphasis on separating peer-level emotional support from specialized clinical therapy, ensuring volunteers cannot perform clinical interventions without psychologist oversight.
6. **Integrated NGO Referrals:** Direct referral pipelines to specialized partner NGOs (e.g., trauma units, rehab centers) are critical when a client requires specialized care beyond standard platform sessions.

Partner NGOs

1. Overview

1. **Interviewee(s):** Representatives from Mental Health NGOs, Crisis Helpline Providers, and Psychological Care Centers
2. **Interviewer(s):** Dipti Sorkar (Bangladesh Mohila Porisod)
3. **Purpose of Interview:** Understand how mental health service providers and partner NGOs currently operate, the barriers individuals face in locating verified psychological support during crises, their willingness to be included in a verified partner directory, and the practical requirements for accurate service listings, real-time availability updates, referral handling, and secure data sharing within the **MindEase** platform.

2. Key Objectives Discussed

- How individuals currently discover that a facility or hotline offers mental health support or crisis shelter during emergencies.
- Barriers that prevent people from finding or reaching mental health institutions quickly in a crisis.
- Value of inclusion in a public, trusted directory of verified mental health centers and partner NGOs that appears on platform resource maps.
- Critical facility details (operating hours, specialized counseling services, crisis numbers, counselor availability) that must remain accurate for users in urgent need.
- Existing coordination with community mental health initiatives or digital platforms, and openness to structured partnerships.
- Concerns about public listing as a crisis support resource and client confidentiality maintenance.
- Processes for handling external referrals from peer volunteers and psychologists, and confirming case urgency.
- Preferred methods for updating resource availability, counselor slots, or operational hours so users see accurate information.
- Experiences with individuals seeking emergency clinical care with incorrect expectations about available services.
- Acceptable data-sharing, privacy compliance, and partnership arrangements.
- Preferred channels for receiving emergency crisis referrals from the platform.
- Desired role of technology in helping individuals find and reach mental health support seamlessly.
- Management of support request surges during high-risk periods (nighttime or high-stress events).
- Feedback mechanisms that would build trust in directory listings and partner credentials.
- Speed required for reflecting changes in facility location, emergency hotline numbers, or contact details.

3. Findings / Key Points

1. Many individuals currently discover mental health facilities only through word of mouth, so a significant number never find appropriate help during severe emotional crises.

2. The main barrier is the lack of accurate, up-to-date service information, emergency hotline contacts, and counselor availability listed in one trusted place.
3. Outdated or incomplete public information regularly causes individuals to seek help during off-hours or with wrong expectations about the facility's clinical capabilities.
4. Coordination with external community health initiatives or crisis apps is currently limited or informal (or absent), creating a need for standardized referral handoffs (such as MindEase NGO referrals).
5. Managing surges in support requests during high-risk periods (nighttime or widespread distress events) often lacks a dedicated triage process or advance visibility of institutional capacity.

